Surgical options for Stress Urinary Incontinence

You are asked to carefully read this information leaflet and consent form to enable you to better understand the procedures available to treat your present symptoms. Should you require clarification or additional information, please notify your doctor who will discuss any questions or concerns you may have. It is a requirement that a consent form be signed before carrying out any surgical procedure.

This information leaflet talks about each type of surgery: how it is performed, the risks, the success rates and how the surgeries compare to each other. If you decide you would rather not proceed with surgery please notify your surgeon and other treatment options will be discussed with you.

Patient Details

First Name:	Barry	Last Name:	oreilly
Date of Birth:	10/01/1966	MRN/Identifier:	
Consultant:			

What is stress urinary incontinence?

Stress urinary incontinence is the leakage of urine during activities such as coughing, sneezing, lifting, laughing or exercising. It affects up to 1 in 5 of women.

Stress urinary incontinence has many causes including: pregnancy, childbirth, obesity, chronic cough, constipation, heavy lifting and genetically inherited factors.

Non-surgical treatment options

It is important to have tried conservative non-surgical options prior to considering surgery. These include:

Lifestyle changes

Weight loss is an effective treatment option for overweight women with stress urinary incontinence.

Fluid reduction to 1-1.5 litres per day can improve symptoms.

Absorbent products such as incontinence underwear or pads may provide additional options for managing urinary issues for some women.

Pelvic floor muscle exercises

Pelvic floor muscle training under the supervision of a specialist physiotherapist can be an effective non-surgical option and should be carried out before surgery. Many women who have undergone pelvic floor physiotherapy will not require surgery.

Continence pessaries and garments

These are devices placed inside the vagina to support the bladder neck and can be effective for managing stress urinary incontinence. Some engineered garments pull the pelvic floor upwards and can help compress the bladder neck and reduce stress incontinence.

Duloxetine (medication)

This is a medication which may improve stress urinary incontinence symptoms. Some side effects of the medication are not tolerated by women. It is recommended as a third line option, if you do not want or cannot have surgery.

Do nothing

No treatment is always an option.

If non-surgical treatment options have not been successful or are not appropriate/suitable, surgical options can be considered.

What types of surgery are available?

Midurethral sling operations

A vaginal operation to stabilise the urethra (waterpipe) using a strip of mesh, sometimes called a tape. This mesh is made of synthetic suture material and stays in your body permanently.

- The retropubic tape operation (TVT) involves making a small incision in your vagina and two small incisions in your lower abdomen just above your pubic bone.
- The transobturator tape operation (TVT-O) involves making a small incision in your vagina and two small incisions on your inner thigh on both sides.
- Rarely, a sling/tape procedure may be made more complicated because of previous pelvic surgery or radiotherapy to the pelvis.

Mesh is a graft material that is woven using medical grade polymer called polypropylene which has been widely used as a suture material in all areas of surgery for more than 50 years and which has an excellent safety profile. This kind of mesh is commonly used for abdominal and groin hernia repairs.

Fascial (natural tissue) sling procedure

Abdominal procedure (open) to lift the urethra (waterpipe) using a strip of connective tissue harvested from your own abdominal wall or from the outside of your thigh. A bikini line incision is required to harvest this tissue. A small incision is made in your vagina. The strip of tissue is used to lift the urethra (waterpipe) and is attached to your abdomen using synthetic stitches. Both permanent and non-permanent sutures can be used.

Colposuspension procedure

Abdominal operation (open or key-hole) where synthetic stitches are placed on either side of the urethra (waterpipe). Both long-lasting absorbable and permanent sutures can be used.

Urethral bulking agents

Vaginal operation where a synthetic 'bulking' material is injected at the bladder neck to improve the seal of the bladder so it is harder for urine to leak out. This is carried out through a camera called a cystoscope which is passed into the urethra (waterpipe) and allows injections to be made.

How do the different types of surgery compare

Each surgery has different short and long term success rates and as with every surgery can be associated with potential complications. The surgical options available to you will be affected by previous surgeries you have had, your weight and your medical history.

Below you will find tables comparing each procedure. At the end of this leaflet you can write down if you have any questions or what your impression of each procedure is. If you have other questions about a procedure please ask your surgeon so that you can make an informed choice.

	Retropubic midurethral sling	Fascial sling	Colposuspension	Urethral bulking agents
How is this procedure performed	A small cut is made in the vagina below your urethra (waterpipe) and in your lower abdomen or inner thigh	This is an open surgery where a bikini line incision (cut) is made and a small cut is made inside your vagina.	This can be done as: an open surgery where a bikini line incision (cut) is made, or keyhole (laparoscopic) surgery - small incisions in your lower abdomen	This is carried out using a cystoscope (small camera passed into the urethra)
Anaesthesia	General, spinal or local anaesthetic with sedation	General or spinal anaesthetic	General or spinal anaesthetic	General, spinal or local anaesthetic with sedation
Day case or hospital stay	Day case or overnight stay	1-3 days in hospital	1-2 days in hospital	Day case or outpatient
Recovery	2 weeks	6 weeks	2-6 weeks	1-2 days

Success rates

	Retropubic midurethral sling	Fascial sling	Colposuspension	Urethral bulking agents
Short-term	80-90% of patients feel their incontinence is either cured or much better	80-90% of patients feel their incontinence is either cured or much better	80% of patients feel their incontinence is either cured or much better after one year	60-70% of patients feel their incontinence is either cured or much better. The effect reduces over time and more than one third of patients require a second injection
Long-term (20 years)	80-90% satisfaction is maintained	70-80% are satisfied with their outcome	60-70% are satisfied with their outcome	

Are there any risks?

The table below is designed to aid you in understanding the risks associated with surgical procedures. Risk can be explained using both words or numbers, or both. The table below has been recommended as a way of describing risk in healthcare. Further explanation on risks in healthcare is outlined by the Royal College of Obstetricians and Gynaecologists "Understanding how risk is discussed in healthcare".

https://www.rcog.org.uk/for-the-public/browse-all-patient-informationleaflets/understanding-how-risk-is-discussed-in-health-care-patient-information-leaflet/

Your individual risk

Certain factors can increase your risk profile. Such factors include:

- Medical conditions such as diabetes
- Smoking
- Taking blood thinning medication
- Being overweight
- If you have had previous surgery for prolapse or incontinence
- Previous radiotherapy

Please discuss your own individual risk with your surgeon.

General complications of pelvic surgery

All surgical procedures carry risks.

General complications of pelvic surgery include:

- Injury to internal organs: rare risk of damage to internal organs requiring further surgery - bladder, ureters (kidney tubes), urethra (waterpipe), bowel and blood vessels
- Bleeding - major bleeding during or after the surgery is uncommon but may require a blood transfusion in some cases (<1/100). Occasionally a haematoma/collection of blood may occur which may require further surgery or time to resolve
- Blood clot: a clot in the deep veins of the leg can occur after surgery in 4-5% of women however the majority go unnoticed and resolve spontaneously. Rarely (<1/100) a clot can pass from the leg to the lungs which is very serious. It is very rare for this to cause death but compression stockings and blood thinning injections are provided after major surgery or to high risk women to prevent this
- Infection is common with any surgery and antibiotics will be given during the surgery to reduce this risk. Wound infections are uncommon but urinary tract infections are common after surgery
- There are also rare individual risks with a general or spinal anaesthetic which are outlined pre-operatively by the anaesthetist
- Death - very rare.

Procedure specific risks

Retropubic and transobturator midurethral sling

Complication	Risk
Mesh exposure in the vagina	Common (1-2/100) risk of mesh becoming infected or rejected resulting in exposure into the vagina. This can happen many years after surgery and can cause bleeding or pain/irritation for you or your partner with sex. This may resolve with local oestrogen therapy or may require partial excision of the mesh
Mesh exposure into the bladder or urethra (waterpipe)	Rare (<1/100). Can occur soon or years after surgery. This can happen if the bladder or urethra are damaged during the surgery and this is not recognised or if the tape migrates years after the surgery. This requires surgery to remove the tape.
Bladder or urethral injury	Common (5-10/100). When discovered during the procedure, the trocar/tape is removed and replaced correctly. The bladder is usually drained with a tube for 24 hours to allow the hole in the bladder to heal. No long term issues have been found following this complication. Damage to the urethra (waterpipe) is uncommon (<1/100) but may have long term consequences
Needing to pass urine more frequently than usual or not reaching the toilet on time	New onset urinary frequency and/or associated leakage with urgency is common (5-10/100). This can be treated with bladder retraining and physiotherapy, and some women need medication. If you have pre-existing overactive bladder your urgency and/or associated leakage with urgency may improve or worsen after surgery. Stress incontinence surgery does not cure urgency symptoms or treat bladder pain.
Temporary difficulty passing urine (voiding dysfunction)	Common (4/100). Women can have problems emptying the bladder fully which is usually resolved in 7-14 days. It may require a short term tube in the bladder (catheter) for a few days. If this persists reoperation may be required to loosen or divide the mesh. Long term voiding dysfunction requiring self-catheterisation for months/years is rare. Long-term voiding dysfunction may be associated with recurrent urinary tract infections.
Temporary vaginal/pelvic pain or pain with sex	Uncommon (1/100). Women can develop pain and this usually resolves spontaneously with pain relief after 1-2 weeks. Pain rarely persists.
Long-term vaginal/pelvic pain or pain with sex	Uncommon with retropubic midurethral sling. *Common (5-10/100) with a transobturator tape and can affect the groin and/or inner thigh. This may be as a result of nerve irritation or muscle spasm. Physiotherapy in the form of "trigger point release" may be helpful and referral to a pain specialist may be required. Full or partial removal of the sling may be needed.

Risks of removing the mesh tape

It may not be possible to safely and completely remove the mesh implant as it is meant to incorporate permanently into your tissues. Complete mesh removal may be associated with higher risks of nerve and organ damage and poor outcomes in terms of pain and continence. Complete removal of the transobturator mesh tape may not be possible.

Referral to a mesh centre (with a multidisciplinary surgical team experienced in mesh removal) may be required. Complete removal of the mesh tape may not alleviate all symptoms and some symptoms may worsen.

Partial and complete removal of mesh tape may cause your stress urinary incontinence to return and you may need to consider another surgery for incontinence.

Fascial sling

Complication	Risk
Fascial exposure in the vagina	Uncommon (<1/100). Risk of fascia becoming infected or rejected resulting in exposure into the vagina. This can cause bleeding or pain/irritation with sex. This may require partial excision of the sling.
Bladder or urethral injury	Common (5-10/100). When discovered during the procedure, the trocar/tape is removed and replaced correctly. The bladder is usually drained with a tube for 24 hours. Damage to the urethra (waterpipe) is uncommon (<1/100) but may have long term consequences
Needing to pass urine more frequently than usual or not reaching the toilet on time	New onset urinary frequency and/or associated leakage with urgency is very common (10/100). This can be treated with bladder retraining, physiotherapy and some women need medication. If you have pre-existing overactive bladder your urgency and/or associated leakage with urgency may improve or worsen after surgery.
Temporary difficulty passing urine (voiding dysfunction)	Very common (10/100). Women can have problems emptying the bladder fully which is usually resolved in 6-8 weeks. It may require a short term tube in the bladder (catheter) for a few days or weeks. If this persists you may need to learn to self-catheterise or reoperation may be required. This is a common (5-10/100) long term risk.
Vaginal/pelvic pain or pain with sex	Common (1/100). Women can develop pain and this usually resolves spontaneously with pain relief after 1-2 weeks. Pain rarely persists and further treatment with physiotherapy, pain management or full or partial removal of the sling may be needed.
Wound complications	Common (2/100). The abdominal wound can become infected or can open up if the stitches become loose. A collection of blood or fluid may form below the incision. This may need antibiotics and/or drainage. Common (2/100) women can develop pins and needles or numbness around the scar.
Hernia	Common (up to 10/100). This can happen at the scar at your bikini line and may require further surgery to repair.

Colposuspension

Complication	Risk
Bladder or urethral injury	Uncommon (<1/100). Rarely the stitches placed may erode into the bladder and require removal. Damage to the urethra (waterpipe) is uncommon (<1/100) but may have long term consequences
Needing to pass urine more frequently than usual or not reaching the toilet on time	New onset urinary frequency and/or associated leakage with urgency is very common (15-20/100). This can be treated with bladder retraining, physiotherapy and some women need medication. If you have pre-existing overactive bladder your urgency and/or associated leakage with urgency may improve or worsen after surgery.
Temporary difficulty passing urine (voiding dysfunction)	Very common (10/100). Women can have problems emptying the bladder fully which is usually resolved in 6-8 weeks. It may require a short term tube in the bladder (catheter). If this persists you may need to learn to self-catheterise. Long-term voiding dysfunction may be associated with recurrent urinary tract infections.
Vaginal/pelvic pain or pain with sex	Common (1-5/100). Women can develop pain and this usually resolves spontaneously with pain relief after 1-2 weeks. Pain rarely persists and further treatment with physiotherapy, pain management or releasing the stitches may be needed.
Wound complications	Common. The abdominal wound can become infected or can open up if the stitches become loose. This may need antibiotics and pain relief.
Prolapse of the back vaginal wall (rectocele)	Very common (15/100). This may present with a bulge or the sensation of something coming down and surgery to repair this may be required.
Problems with stitches placed	Rare. If the stitches that are used are permanent they can erode through to the bladder or the vaginal wall.

Urethral bulking agents

Complication	Risk
Temporary difficulty passing urine (voiding dysfunction)	Common (<10/100). Women can have problems emptying the bladder and may require a short term tube in the bladder (catheter) for one or two days. It is rarely more prolonged than this. Long term voiding dysfunction is very rare.
Pain passing urine	Common (1/100). Pain or stinging passing urine can occur in the first 24-48 hours. If you develop symptoms of a urinary tract infection you will need antibiotics.
Allergy/hypersensitivity to the bulking material	Uncommon (<1/100). Rare occurrence but may require treatment for hypersensitivity
Abscess (local infection) or granuloma (cyst like structure)	Uncommon (<1/100). Rare but may need treatment with antibiotics or excision of the granuloma.
Need for repeat bulking injection	A "top-up" can be required to successfully treat symptoms of stress urinary incontinence. The effect of the bulking material may sometimes reduce with time requiring a second injection.

What preparation is advised before surgery

- Stopping smoking several weeks before surgery reduces your chances of complications such as infection, blood clots and poor healing and improves your overall health.
- You may need to stop taking blood thinning medication such as Aspirin, Plavix (clopidogrel), warfarin (coumadin) or Xarelto (rivaroxaban) - please inform the consulting doctor if you are taking any of these.
- Try to maintain a healthy weight.
- Optimise your blood sugars if you are diabetic.
- Avoid constipation.

What to expect after surgery

Depending on the surgery you have you may or may not be kept in hospital. Your recovery after the surgery will be monitored and it is usual to be allowed to eat and drink. Ward staff will monitor the amount of urine you pass and scan your bladder after voiding to ensure you are emptying your bladder.

The first few days/weeks

- There might be some vaginal bleeding and if you need to wear protection use a sanitary pad, not a tampon.
- You can drive as soon as you can push the pedals and look over your shoulder without discomfort - usually after two or three weeks. You need to check this with your insurance company.
- There is no restriction on undertaking light activities in the first few days if you feel comfortable to do so. After six weeks gradually build up your level of activity. More strenuous tasks and heavy lifting should be avoided for six weeks. After 3 months you can return to your usual level of activity.
- You should refrain from sexual intercourse and inserting any creams or devices into the vagina for 6 weeks following your procedure, unless recommended by your doctor.
- It is important that you avoid constipation by ensuring you drink plenty of fluid and eat fruit, vegetables and roughage (e.g. bran/oats) high in fibre. Laxatives may be required to make your bowels work better.
- Return to work will depend on the type of work you do. Please ask your doctor for his/her opinion and if you require a "Fitness for work" certificate.

Pregnancy and childbirth

It is highly advisable that you wait until your family is complete before considering any surgery for stress urinary incontinence. Carrying a pregnancy and having a vaginal delivery may increase the risk of failure of your incontinence procedure. If you do become pregnant a caesarean section may be recommended for your delivery in order to reduce this risk.

Questions and Expectations

Please write down any questions you have below and bring this booklet with you to your next appointment with your clinician.

Things I would like to know before my operation. Please list below any questions you may have, having read this leaflet:

1.
2.
3.

Please document your expectations of this treatment

Patient Request for TVT Insertion

Patient request for retropubic mesh tape (TVT) for the treatment of debilitating stress urinary incontinence

> Important: This procedure has been put on pause by HSE Ireland since 2018 and is only currently permitted in exceptional circumstances.

I wish to proceed to the insertion of a TVT retropubic mesh sling to try to help my ongoing stress incontinence symptoms. I can confirm that:

- x 1. I have daily debilitating stress urinary incontinence affecting my physical and mental wellbeing.
- x 2. I am aware that this procedure has been put on pause by HSE Ireland since 2018 and is only currently permitted in exceptional circumstances.
- x 3. I have received lifestyle advice.
- x 4. I have undertaken supervised pelvic floor muscle training.
- x 5. I do not wish to receive any further non-surgical treatments such as physiotherapy, pessary, or vaginal devices as I have considered these and/or not found them to be beneficial.
- x 6. I have carefully read and considered all the surgical options available to treat this condition.
- x 7. I have received the Patient Decision Aids introduced by NICE for the management of women with Stress urinary incontinence.
- o 8. I have considered/had a Urethral Bulking agent and do not wish to have this performed/repeated. I understand Urethral Bulking agent would be a minor outpatient procedure.
- x 9. I decline to have either a laparoscopic/open colposuspension or an autologous fascial sling. I personally do not accept the risk profile of these procedures.
- x 10. I believe that the risk profile of a retropubic mesh tape is more acceptable to me in my own circumstances.
- x 11. I understand that the mesh used for a TVT is a type 1 Polypropylene mesh and is permanent and is not intended for removal.

Risk Acknowledgement

I have received patient Information leaflets (PIL) for the TVT and am aware of the following risks:

- Cure/improvement rates: 80-90% (a 10-20% failure rate)
- Recurrence of incontinence symptoms: 20-30% over the next 10-20 years
- Infection: less than 5%
- Bleeding: may require further surgery and potential blood transfusion (less than 5%)
- Blood clots in legs (DVT) and lungs (PE): less than 1%
- Injury to nerves, bladder, bowel, and blood vessels: less than 5%
- Chronic pain: less than 5%
- New onset or worsening of overactive bladder symptoms (frequency, urgency, urge leakage): up to 7%
- Difficulty emptying bladder: 5%
- Need for self-catheterisation short and long term: less than 5%
- Exposure of mesh through vaginal wall: up to 5%
- Need for mesh removal due to complications: up to 3%. Complete removal may not be possible and may not resolve mesh-related symptoms.
- Sexual dysfunction: up to 15%, which may include pain during intercourse

x I am fully aware of the success rates and complications of these procedures, and I have had time to understand these. I have carefully read the section on mesh complications and accept these complications.

o I am aware of the adverse publicity associated with the use of synthetic vaginal mesh and the concerns raised by campaign groups.

MDT Approval Requirement

x I am aware that it is necessary for my case to be approved by the local Urogynaecology Multidisciplinary team meeting (MDT). This will comply with my ability, as the patient, to exercise my right to informed patient choice.

Irish National Mesh Register

I have read the Irish National Mesh Register patient information leaflet and/or had it explained to me. I understand the reasons for the Irish National Mesh Register, that it aims to record all mesh implants and improve care delivered to patients in Irish hospitals.

Consent to the Irish National Mesh Register

x I CONSENT to my details being recorded on the Irish National Mesh Register. I understand that I may withdraw this consent at any time in the future.

Mesh Device Information

Device Name: ______________________________

Manufacturer: ______________________________

Consent Form

This consent must be completed by the doctor with their patient in advance of surgery. All sections must be signed.

Section A: Non-surgical Management Discussion

My clinician has discussed with me the benefits of non-surgical management of stress urinary incontinence including lifestyle interventions, physiotherapy and behavioural therapies.

Patient Signature

Doctor's Signature

Section B: Physiotherapy Attendance

o I have attended a physiotherapist who has explained to me non-surgical options for stress urinary incontinence. My wish is to proceed with surgical intervention.

Section C: Urodynamics Attendance

o I have attended a urodynamics clinic. My wish is to proceed with surgical intervention.

Section D: Patient Expectations/Hopes

What are you hoping the operation will do? Please describe the symptoms you think this surgery will cure - please list all:

Section E: Information Leaflet Confirmation

x I confirm I have been given an information leaflet to read at home prior to my surgery date. I understand the risks that have been explained and I still wish to proceed with surgery.

Section F: Information Checklist

I confirm I have had adequate time to study the information provided. I am satisfied with the explanation of the procedure and the associated risks and benefits that have been discussed with me. I have no further questions.

x The details of the procedure proposed and the desired outcome

x All available alternatives and their advantages and disadvantages

x All information on possible risks including my individual risk

x All my questions were answered

Statement of Health Professional

- I am suitably trained and competent and have sufficient knowledge to consent this patient in line with the requirements of my regulatory body.
- I have discussed what the treatment is likely to involve, the benefits and risks of this procedure.
- I have also discussed the benefits and risks of any available alternative procedures or treatments including no treatment.
- I have discussed any particular concerns of this patient.

Name:	BOR	Job Title:	Consultant
Signature:	Charles Leahy	Date:	08/02/2026

Patient Declaration

My health professional has presented all the evidence and data on stress urinary incontinence surgery. Having read the available literature (PIL) and this form, I confirm that I understand the risks and, on the basis of my own wishes, want to proceed with the TVT.

I have the right to change my mind at any time, including after I have signed this form.

x I confirm that there is no risk that I could be pregnant.

Patient Name:	Barry oreilly	Date:	08/02/2026

Patient Signature

MDT Approval (For Clinical Use)

Approved to pass to MDT:			
Consultant Name:		Signature:	
Date:			

Additional Resources

BSUG Patient Information: https://bsug.org.uk/pages/for-patients/bsug-patient-information-leaflets/154

RCOG Mid-urethral Sling Information: https://www.rcog.org.uk/en/patients/patient-leaflets/mid-urethal-sling-operation-for-stress-urinary-in-continence/